

ROYAL FITNESS CLUB

RFC PREMIUM GUIDE #10

ULTIMATE FAT LOSS MASTERCLASS

Complete Science-Based Fat Loss System

6 Chapters · Science-Based · All Levels · Hormone-Aware

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DISCLAIMER

This guide is for educational and harm-reduction purposes only. It does not constitute medical advice. Consult a qualified healthcare professional before starting any supplementation or pharmaceutical protocol. Individual results vary.

CHAPTER 1 — ENERGY BALANCE SCIENCE

"Fat loss is ultimately a physics problem — but physiology makes it a non-linear, adaptive, hormonal physics problem. Treat it accordingly."

The first law of thermodynamics applies: fat loss requires a sustained caloric deficit. However, the human body is not a static furnace — it is an adaptive system that responds to caloric restriction by reducing total daily energy expenditure (TDEE) through multiple pathways. Understanding these adaptations is what separates effective long-term fat loss strategies from failed crash diets.

TDEE Component	Contribution	Adaptability	Implication
Basal Metabolic Rate (BMR)	60–70%	Decreases with muscle loss + adaptation	Preserve muscle — it drives BMR
Thermic Effect of Food (TEF)	8–15%	Decreases with caloric deficit	High protein diet maximises TEF
Non-Exercise Activity (NEAT)	15–30%	High — unconsciously reduces in deficit	Stay active outside gym; walk more
Exercise EE	5–15%	Stable if intensity maintained	Maintain training intensity in cut

■ *The biggest threat to fat loss is NEAT reduction — your body unconsciously moves less when in deficit. Deliberate daily step targets (8,000–12,000 steps) counteract this adaptation.*

CHAPTER 2 — HORMONES, METABOLISM & ADAPTATION

Hormone	Fat Loss Role	Changes in Deficit	Management Strategy
Leptin	Satiety, metabolic rate regulator	Drops within days of deficit	Diet breaks, refeeds
Ghrelin	Hunger hormone	Increases in deficit	High fibre, protein, structured meal timing
Insulin	Fat storage gate	Improves with deficit	Lower carb diet amplifies fat oxidation
Cortisol	Stress hormone, fat storage (visceral)	Elevated in severe deficit	Moderate deficit; sleep priority
T3 (thyroid)	Metabolic rate	Decreases with severe restriction	Avoid extreme deficits > 25% TDEE
Testosterone	Anabolic, anti-fat	Decreases in large deficit	Moderate deficit, adequate fat intake

The Diet Break Protocol

A diet break (7–14 days at maintenance calories) every 8–12 weeks restores leptin, normalises ghrelin, reduces cortisol, and resets metabolic rate. Research shows that dieters using diet breaks lose the same amount of fat over time as continuous restrictors, while preserving significantly more muscle and maintaining better hormonal health.

CHAPTER 3 — OPTIMAL FAT LOSS NUTRITION

The Non-Negotiable Fat Loss Rules

- Caloric deficit: 300–500 kcal/day below TDEE. Start conservative — deepen only when fat loss stalls
- Protein: 2.0–2.7g/kg bodyweight — highest priority macronutrient for muscle preservation
- Fat minimum: 0.7–1.0g/kg bodyweight — critical for hormonal health, especially testosterone
- Carbohydrates: the flexible variable — fill remaining calories after protein and fat targets met
- Fibre: 30–40g/day minimum — satiety, gut microbiome, appetite regulation
- Water: 3–4L/day — research shows optimal hydration improves fat oxidation by 30%

Day Type	Calories	Protein	Carbs	Fat	Best For
Training day	TDEE – 200	2.2g/kg	3g/kg	0.8g/kg	Fuel + recovery
Rest day	TDEE – 500	2.5g/kg	1.5g/kg	1.0g/kg	Deeper deficit
High carb (weekly)	TDEE + 100	2.0g/kg	4g/kg	0.6g/kg	Leptin reset, performance

CHAPTER 4 — CARDIO SCIENCE

Method	Intensity	Duration	Fat Oxidation	Best Application
LISS (Low Intensity Steady State)	Zone 2 (60–70% HRmax)	40–60 min	High during session	Daily calorie burn, recovery
HIIT (High Intensity Interval)	>85% HRmax intervals	15–25 min	High (24h EPOC)	2–3× per week, metabolic
Zone 3 (Moderate)	70–80% HRmax	30–45 min	Moderate	Aerobic capacity building
Incline Walking	5–7% grade, 5–6 km/h	45–60 min	High, joint-friendly	Daily non-disruptive cardio

■ *HIIT burns more total calories in less time and produces EPOC (excess post-exercise oxygen consumption) lasting up to 24 hours. However, it stresses the CNS — limit to 2–3 sessions/week maximum alongside resistance training.*

CHAPTER 5 — RESISTANCE TRAINING FOR FAT LOSS

"Cardio creates the deficit. Resistance training determines what fills the resulting space — muscle or fat. Without resistance training, fat loss is partly muscle loss."

- Maintain all heavy compound lifts through a cut — reduce volume before reducing intensity
- Train with the same weight as your bulk when possible — this signals "need this muscle"
- Superset non-competing muscle groups to increase calorie burn without extending training time
- Track strength closely — a 10–15% strength reduction is acceptable; more indicates muscle loss
- Abs and core training: every session; low body fat reveals abs — they are built through compound training

Fat Loss Training Split

Day	Session	Duration	Cardio After?
Mon	Heavy Lower Body (Squat, RDL, Leg Press)	50 min	20 min LISS
Tue	Heavy Upper Push (Bench, OHP, Dips)	50 min	No — recovery
Wed	HIIT + Core	35 min	HIIT is cardio
Thu	Heavy Upper Pull (Row, Pullup, Face Pull)	50 min	20 min LISS
Fri	Metabolic Circuit (full body, light)	45 min	Optional LISS
Sat/Sun	Walk 8,000+ steps, mobility, rest	60 min walk	Walking IS cardio

CHAPTER 6 — 12-WEEK FAT LOSS BLUEPRINT

Phase	Weeks	Deficit	Cardio	Focus
Phase 1: Foundation	1–4	300–350 kcal	3× LISS 30 min	Habit formation, baseline
Phase 2: Acceleration	5–8	400–450 kcal	4× LISS + 1× HIIT	Increase intensity, deepen cut
Phase 3: Peak	9–11	450–500 kcal	4× LISS + 2× HIIT	Maximum fat mobilisation
Phase 4: Recomp	12	Maintenance	2× LISS	Diet break, hormonal reset

■ *Expected total fat loss over 12 weeks following this blueprint: 4–6kg of pure body fat. Scale may show more due to water loss — body fat % and measurements tell the true story.*